

Mobilize & go-decision packet

Ambient Clinical Value Chain Activation

Tenant: Meridian Health System

Tenant key: meridian-health

Move ref: meridian:move:ambient-clinical-value-chain-activation

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The mobilization plan, adoption & change approach, the value-measurement model handed to Tower and the go-decision pack.

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Mobilize & Handoff — adoption, measurement, go-decision

Ambient Clinical Value Chain Activation · meridian-health

Go-decision: NO GO

No-go: 2 Mobilize kill trigger(s) fired (measurement_unwired, business_case_blocker_open). The Move is not ready to leave Moves. See the fix-conditions for what must change before it is reconsidered.

Mobilize readiness

Owned	Yes
Adoptable	Yes
Measurable	NO
Operating-model owner	Dr. Morales (CMIO, Clinical Informatics)
Overall change load	high

Fired kill triggers

- measurement_unwired: 1 committed value metric(s) cannot be wired to a recorded baseline: Cost-per-clinician-hour conversion coefficient. — fix: Revisit when every committed metric is wired to a Discover baseline value or an owned, dated plan to capture one.
- business_case_blocker_open: The business-case critic has 1 open blocker(s): cfo_monetisation_blocked. — fix: Revisit when the business-case critic reports no open blocker.

Conditions to clear

- measurement_unwired: Revisit when every committed metric is wired to a Discover baseline value or an owned, dated plan to capture one.
- business_case_blocker_open: Revisit when the business-case critic reports no open blocker.

Adoption & change approach — seven dimensions

Dimension	Magnitude	Recommendation
impacted_roles	high	Three roles change; the clinician role changes most. Stage the rollout by specialty, leading with primary care where pajama time is highest.
process_variance	high	Specialty template governance varies documentation materially; the process-redesign workstream must land before the change rollout (it is budgeted in the Design & Plan estimate).
training_load	moderate	Role-based training: ambient-tooling curriculum for clinicians, a queue-triage curriculum for CDI specialists.
incentive_change	moderate	Re-point clinician scorecards away from raw note volume toward documentation quality and recovered capacity.
manager_adoption	high	Manager reinforcement is the highest-risk layer. A dedicated clinic-manager enablement track with explicit reinforcement in the hypercare window.
communications	moderate	Address the clinician narrative directly — ambient documentation recovers cognitive load and after-hours time; it is not a surveillance layer.
hypercare	moderate	An 8-week hypercare window with elevated informatics support and a weekly documentation-quality review; exit when pajama time and CDI volume hold for two consecutive weeks.

Impacted roles

Role	Headcount	What changes
Primary-care clinician	Not recorded — seed gap	Clinicians move from manual after-hours documentation to ambient-captured notes reviewed and signed in-visit.
CDI specialist	42	CDI specialists work an ambient-routed HCC query queue rather than a starved, manually-built one.
Clinic operations manager	Not recorded — seed gap	Managers coach to the new documentation pattern and the recovered-capacity visit targets.

Value-measurement model — handed to Tower

Metric	Baseline	Target	Owner
Primary-care pajama time	129	100	CMIO
CDI HCC-relevant query volume	40	160	CDI Director

HCC capture on the MA panel

58	63	VP Revenue Cycle	
Cost-per-clinician-hour conversion coefficient	Not recorded — seed gap	Not recorded — seed gap	VP Revenue Cycle

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